OUTPATIENT IMAGING

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60 Oak Hill Blvd #101 • Newnan • GA • 770-502-9883
2284 Peachtree Rd NW • Atlanta • GA • 404-500-1658
1233 HWY 54 Suite 110 • Fayetteville • GA • 770-288-0082

Patient Name: **MITCHELL, GERARD** **Sex:** **M**
Date of Birth: **28-Mar-1977** **Exam Date:** **08-Nov-2024 05:30:00 PM**
Patient Number: 5379278
Referring Physician: **DEARBORN, MARY**

EXAM: MRI OF THE CERVICAL SPINE WITHOUT GADOLINIUM CONTRAST:

CLINICAL INDICATION: The patient has a history of an injury on 10/18/24 with complaints of new onset cervicalgia subsequent to the injury. No history of related surgery is provided. ICD 10 code is M54.2.

COMPARISON: No relevant studies for comparison.

TECHNIQUE: Sagittal T1 and T2-weighted and axial gradient recall images through the region of the cervical spine were obtained and submitted for interpretation. No plain film images are available for correlation at the time of this dictation. Gadolinium contrast was not administered ordered by the referring physician. No previous studies or reports of previous studies are available for comparison or correlation at the time of this dictation.

FINDINGS: The cerebellar tonsils have a normal anatomic relationship with the foramen magnum. The odontoid is intact. The vertebral body heights are maintained. There is reversal of the normal cervical lordosis with an axis at C3-C4.

At C2-C3, there is a broad-based bulge of the intervertebral disc without neural foraminal narrowing.

At C3-C4, there is a posterior herniation of the intervertebral disc effacing the thecal sac and distorting the cord with right and more severe left neural foraminal stenosis.

At C4-C5, there is a midline bulge of the intervertebral disc effacing the ventral aspect of the thecal sac.

At C5-C6, there is a posterior herniation of the intervertebral disc effacing the thecal sac with right neural foraminal stenosis.

At C6-C7, there is a posterior herniation of the intervertebral disc with bilateral neural foraminal stenosis effacing the thecal sac and distorting the cord.

At C7-T1, there is no evidence of neural foraminal narrowing or central spinal canal stenosis.

IMPRESSION:

OUTPATIENT IMAGING

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Patient Name: **MITCHELL, GERARD** **Sex:** **M**
Date of Birth: **28-Mar-1977** **Exam Date:** **08-Nov-2024 05:30:00 PM**
Patient Number: 5379278
Referring Physician: **DEARBORN, MARY**

AT C2-C3, BROAD-BASED BULGE OF THE INTERVERTEBRAL DISC WITHOUT NEURAL FORAMINAL NARROWING.

AT C3-C4, POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC EFFACING THE THECAL SAC AND DISTORTING THE CORD WITH RIGHT AND MORE SEVERE LEFT NEURAL FORAMINAL STENOSIS.

AT C4-C5, MIDLINE BULGE OF THE INTERVERTEBRAL DISC EFFACING THE VENTRAL ASPECT OF THE THECAL SAC.

AT C5-C6, POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC EFFACING THE THECAL SAC WITH RIGHT NEURAL FORAMINAL STENOSIS.

AT C6-C7, POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC WITH BILATERAL NEURAL FORAMINAL STENOSIS EFFACING THE THECAL SAC AND DISTORTING THE CORD.

REVERSAL OF THE NORMAL CERVICAL LORDOSIS WITH AN AXIS AT C3-C4.

NO ACUTE FRACTURE OF THE CERVICAL SPINE IS IDENTIFIED.

Electronically signed
DOUGLAS HORNSBY, M.D., F.A.C.R.
DH/vb

OUTPATIENT IMAGING

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1233 HWY 54 Suite 110 • Fayetteville • GA • 770-288-0082

Patient Name: **MITCHELL, GERARD** **Sex:** **M**
Date of Birth: **28-Mar-1977** **Exam Date:** **08-Nov-2024 05:08:26 PM**
Patient Number: 5379278
Referring Physician: **DEARBORN, MARY**

EXAM: MRI OF THE LUMBAR SPINE WITHOUT GADOLINIUM CONTRAST:

CLINICAL INDICATION: The patient has a history of an injury on 10/18/24 with complaints of new onset low back pain subsequent to the injury. No history of related surgery is provided. Concern is for ligamentous injury of the lumbar spine. The patient has a history of lymphoma.

COMPARISON: No relevant studies for comparison.

TECHNIQUE: Sagittal T1 and T2-weighted images were acquired. A set of para-axial T2-weighted images was acquired through the lumbar spine intervertebral discs without gadolinium contrast. For the purposes of this dictation only, there will be the presumption of five lumbar configured vertebrae. No plain films were available for correlation. No previous studies or reports of previous studies were available for comparison or correlation at the time of the interpretation.

FINDINGS: The conus medullaris is normal in signal and morphology and terminates at L1-L2. The paravertebral musculature has a normal MRI appearance. The visualized portions of the abdominal aorta are normal in caliber. Vertebral body heights are maintained.

At L5-S1, there is anterior spurring and modic type 2 changes with a posterior herniation of the intervertebral disc with moderate to severe left neural foraminal stenosis and moderate right neural foraminal stenosis.

At L4-L5, there is anterior spurring and a posterior herniation of the intervertebral disc with moderate bilateral neural foraminal stenosis.

At L3-L4, there is anterior spurring and a posterior herniation of the intervertebral disc with moderate left and right stenosis. The stenosis is right greater than left.

At L2-L3, there is anterior spurring without neural foraminal stenosis.

At L1-L2, there is a posterior herniation of the intervertebral disc paracentral to the left with mild left neural foraminal narrowing.

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Patient Name: **MITCHELL, GERARD** **Sex: M**
Date of Birth: **28-Mar-1977** **Exam Date: 08-Nov-2024 05:08:26 PM**
Patient Number: 5379278
Referring Physician: DEARBORN, MARY

At T12-L1, there is no evidence of neural foraminal narrowing or central spinal canal stenosis.

IMPRESSION:

AT L5-S1, ANTERIOR SPURRING AND MODIC TYPE 2 CHANGES WITH A POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC WITH MODERATE TO SEVERE LEFT NEURAL FORAMINAL STENOSIS AND MODERATE RIGHT NEURAL FORAMINAL STENOSIS.

AT L4-L5, ANTERIOR SPURRING AND A POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC WITH MODERATE BILATERAL NEURAL FORAMINAL STENOSIS.

AT L3-L4, ANTERIOR SPURRING AND A POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC WITH MODERATE LEFT AND RIGHT STENOSIS. THE STENOSIS IS RIGHT GREATER THAN LEFT.

AT L2-L3, ANTERIOR SPURRING WITHOUT NEURAL FORAMINAL STENOSIS.

AT L1-L2, POSTERIOR HERNIATION OF THE INTERVERTEBRAL DISC PARACENTRAL TO THE LEFT WITH MILD LEFT NEURAL FORAMINAL NARROWING.

NO ACUTE FRACTURE OF THE LUMBAR SPINE IS IDENTIFIED.

Electronically signed
DOUGLAS HORNSBY, M.D., F.A.C.R.
DH/vb

MITCHELL, Gerard **DOB:** 03/28/1977 (47 yo M) **Acc No.** 15514 **DOS:** 11/12/2024

MITCHELL, Gerard

47 Y old Male, DOB: 03/28/1977
Account Number: 15514
5780 ORLY TER, ATLANTA, GA-30349-3871
Home: 470-535-6396
Guarantor: Mitchell, Gerard Insurance: POD A
Appointment Facility: Axis Joint And Spine Surgical Center

11/12/2024 **Fitzclarence Harper, MD**

Assessments

1. Radiculopathy, lumbar region - M54.16 (Primary)
2. Sacroiliitis, not elsewhere classified - M46.1

Procedures

Sacroiliac Joint Injection w/o Sedation:

- Indication SI Joint Pain.
- Complications The procedure was performed uneventfully and was well-tolerated by the patient. No adverse events were noted, and the patient was discharged home in stable condition..
- Discharge All potential side effects and adverse events have been discussed with the patient. The patient has been instructed to call my clinic at the first sign of an adverse event, or with any other concerning symptoms.
- Procedure Note Informed consent was obtained for SI joint injections and the patient was taken to the procedure room and placed in the prone position. The patient's low back was prepped with Chlorhexidine and draped in a sterile fashion. Both SI joints were identified by fluoroscopy. The skin and underlying tissues were then anesthetized with 6 ml of 1% Lidocaine and 6 ml of 0.5% Marcaine. I then entered the SI joint space using a 22G 3.5-inch spinal needle. Correct needle tip position was then confirmed by two fluoroscopic views in AP and lateral, and also by injecting 0.5 ml of Omnipaque 240 mg/ml into each space. Bilateral injections were then performed by injecting 4 ml of 0.5% Marcaine and 80 mg of Depomedrol..

Lumbar Transforaminal Epidural Steroid Injection w/o Sedation::

- Indication Lumbar Radiculopathy.
- Complications The procedure was performed uneventfully and was well-tolerated by the patient. The patient was transferred to an observation room and recovered without incident. No adverse events were noted, and the patient was discharged home in stable condition..
- Discharge All potential side effects and adverse events have been discussed with the patient. The patient has been instructed to call my clinic at the first sign of an adverse event, or with any other concerning symptoms..
- Procedure Note After informed consent was obtained, the patient was placed in the prone position on the angiographic table. The back was prepared and draped in normal sterile manner. 2% Lidocaine mixed with equal parts of 0.25% Marcaine was injected for local anesthesia. Under fluoroscopic guidance at the AP, oblique and cross table lateral

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MITCHELL, Gerard **DOB:** 03/28/1977 (47 yo M) **Acc No.** 15514 **DOS:** 11/12/2024

angles, a 22g needle was advanced to the exiting nerve root of L4/5, L5/S1 on the left then right side and a small amount of contrast was injected to confirm needle placement. Following this 1.0 cc 80 mg/ml DepoMedrol and 2.0 cc Marcaine 0.25% were injected per side. The procedure was performed uneventfully and was well-tolerated by the patient. The patient was transferred to an observation room and recovered without incident. No adverse events were noted, and the patient was discharged home in stable condition.

Procedure Codes

- 64483 Transforaminal Lumbar, Modifiers: 50
- 64484 INJ FORAMEN EPIDURAL ADD-ON, Modifiers: 50
- 77003 Fluor Guidance for Spine Injection, Modifiers: 59
- 27096 Sacroiliac Joint Injection, bilateral, Modifiers: 50
- 77002 Fluor Guidance for Injection, Modifiers: 59
- J1040 INJ METHYLPRDNISOLONE ACTAT 80 MG, Units: 3.00
- S0020 Marcaine 0.5% OR 0.25%, Units: 2.00

Follow Up

2 Weeks

Electronically signed by Fitzclarence Harper , MD on 11/12/2024 at 10:35 AM EST

Sign off status: Completed

Axis Joint And Spine Surgical Center
1001 Virginia Ave
Suite 150A
Hapeville, GA 303541328
Tel: 678-782-7999
Fax: 404-334-7274

MITCHELL, Gerard **DOB:** 03/28/1977 (47 yo M) **Acc No.** 15514 **DOS:** 11/11/2024

MITCHELL, Gerard

47 Y old Male, DOB: 03/28/1977
Account Number: 15514
5780 ORLY TER, ATLANTA, GA-30349-3871
Home: 470-535-6396
Guarantor: Mitchell, Gerard Insurance: POD A
Appointment Facility: Verispine Stockbridge

11/11/2024 **Progress Note: Fitzclarence Harper, MD**

MITCHELL, Gerard **DOB:** 03/28/1977 (47 yo M) **Acc No.** 15514 **DOS:** 11/11/2024

Current Medications
None

Past Medical History
- Non-Hodgkins lymphoma- in remission.

Surgical History
- Denies Past Surgical History

Social History
Tobacco Use:
- Tobacco Use/Smoking
  Tobacco use: *nonsmoker*

Drug/Alcohol:
- Drugs
  Have you used drugs other than those for medical reasons in the past 12 months? *No*
- Alcohol Screen (Audit-C)
  Did you have a drink containing alcohol in the past year? *No*
- Caffeine
  Intake: *none*

Miscellaneous:
- Exercise: 1-2 times per week.

Social History:
- Household
  Marital Status: *Single*

Allergies
- N.K.D.A.

Hospitalization/Major Diagnostic Procedure
- Denies Past Hospitalization

Review of Systems
General / Constitutional:
- Patient complains of Headache,Lightheadedness,Sleep disturbance,Weakness.

Musculoskeletal:
- Patient complains of Back pain,Back problems,Limping gait,Neck pain,Weakness.

Reason for Appointment
1. Headaches eith Loss of Conscioudness
2. Neck Pain
3. Low Back Pain
4. Left Leg Numbness & Weakness

History of Present Illness
MVA:

This 47 year old patient presents today wit a complaint of headaches, Loss of Consciousness, Neck Pain and low Bak pain that started after he had a motor vehicle accident that occurred on 10/18/2024. Patient states he was the restrained driver of a Chrysler 300 and was stopped due to traffic on Highway 138 in Clayton County when the driver of a Ford F150 rear-ended him causing him to hit the vehicle in front of him. The patient's car is not totaled. Patient states that he did hit his head on the window and has since been suffering from brain fog and headaches. First responders did come to the scene and had to wake him jup after the collision. However, patient did not go to the ER. Subsequent to the accident tatient has started chiropractic therapy with Iceburg Chiropractic. He is currently undergoing therapy 3 times a week.
Patient states therapy does help, however, it is only short term and then his symptoms return.
Patient complains of neck pain radiating to bilateral upper extremities causing numbness and tingling into the fingers. Patient also states he is unable to keep his neck in one position for prolonged periods of time due to increasing neck spasms, stiffness and radicular symptoms to his upper extremities]. He states he has been experiencing spasms and stiffness on both side of the neck radiating to their mid back and shoulder blades. This neck pain is also worse with lateral rotation ans lateral flexion of his lumbar spine.
Patient also complains of lower back pain radiating to bilateral glutes and lower extremities into the thighs, knees, calves and toes with numbness and tingling into all toes. He also reports persistent left sided numbness with weakness and difficulty with ambulation.
Patient states he is unable to sit, walk, bend, squat, twist or lay in one position for prolonged periods of time due to persistent symptoms in the lateral part of his back. Thsat dpain is worse with transferring form lhying to sitting and sitting to standing, and even worse with the resumption of the upright position after forward flexion of his lumbar spine. he also complaisn of midline low back pain that is worse with lateral flexion and lateral rotation of his lumbar spine. He also reports onset of increased frequency of urination s/p MVA. In addition, his sleep is very poor due to severe pain through the night.
He has had MRIs of the cervical and lumbar spines. Detailed findings are summarized below.

MRI Cervical Spine without Contrast 11/8/24
C2/3 broad-based bulge of the intervertebral disc without neural foraminal narrowing
C3/4 posterior herniation of the intervertebral disc effacing the thecal sac and distorting the cord with right and more severe left neural

foraminal stenosis
C4/5 midline bulge of the intervertebral disc effacing the ventral aspect of the thecal sac
C5/6 posterior herniation of the intervertebral disc effacing the thecal sac with right neural foraminal stenosis
C6/7 posterior herniation of the intervertebral disc with bilateral neural foraminal stenosis effacing the thecal sac and distorting the cord

MRI Lumbar Spine without Contrast 11/8/24
L5/S1 anterior spurring and modic type 2 changes with a posterior herniation of the intervertebral disc with moderate right neural foraminal stenosis
L4/5 anterior spurring and a posterior herniation of the intervertebral disc with moderate bilaeral neural foraminal stenosis
L3/4 anterior spurring and a posterior herniation of the intervertebral disc with moderate left and right stenosis. The stenosis is right greater than left
L2/3 anterior spurring without neural foraminal stenosis
L1/2 posterior herniation of the intervertebral disc paracentral to the left with mild left neural foraminal narrowing.

Vital Signs

BP: **165/113** mm Hg, HR: **131** /min, RR: **19** /min, Temp: **98** F, Oxygen sat %: **97** %, Pain scale: **10** 1-10.
Tigner, Caitlin M 11/11/2024 03:30:22 PM EST >.

Examination

General Examination:

- General appearance: alert, pleasant, well-nourished and in no acute distress, .

Physical Examination

Cervical Spine:

- Range of Motion: Forward Flexion Limited.
- Range of Motion: Extension Limited.
- Range of Motion: Lateral Extension Limited.
- Range of Motion: Rotation Limited.
- Palpation with spinous process tenderness, with muscular tenderness.
- Spurling Test Positive.
- Cervical Facet moderate pain that is worse with with provocative maneuvers bilateral (left > right) C4/5, C5/6, C6/7.
- Trigger Point Location Sub-occipitals, Paracervicals.

Thoracic Spine:

- Range of Motion: Bilateral Upper Extremities Limited.
- Palpation with spinous process tenderness, without musculature tenderness.
- Trigger point location Trapezius, Paraspinal, Rhomboids.
- Facets Pain with provocative maneuvers.

Lumbar Spine:

- Range of Motion: Forward Flexion Limited.
- Range of Motion: Extension Limited.

- Range of Motion: Lateral Extension Limited.
- Palpation with spinous process tenderness, with musculature tenderness.
- Straight Leg Raise Positive, Calf, Heel, with numbness, with tingling.
- Sacroiliac Joint Pain with provocative maneuvers, Patricks, Yoemans, Compression, Thrust.
- Trigger point location Paraspinal, Upper gluteals, Bilateral.
- Sensation Intact Decreased sensory left L4, L5, S1.
- Lumbar Facet moderate bilateral facet pain that is worse with axial loading at L3/4, L4/5 & L5/S1..

Assessments

1. Radiculopathy, lumbar region - M54.16 (Primary)
2. Sacroiliitis, not elsewhere classified - M46.1
3. Radiculopathy, cervical region - M54.12
4. Spondylosis without myelopathy or radiculopathy, lumbar region - M47.816
5. Spondylosis without myelopathy or radiculopathy, cervical region - M47.812
6. Sprain of ligaments of cervical spine, initial encounter - S13.4XXA
7. Sprain of sacroiliac joint, initial encounter - S33.6XXA
8. Sprain of ligaments of lumbar spine, initial encounter - S33.5XXA
9. Postconcussional syndrome - F07.81
10. Post-traumatic headache, unspecified, not intractable - G44.309
11. Other intervertebral disc displacement, lumbar region - M51.26
12. Other intervertebral disc displacement, lumbosacral region - M51.27
13. Other cervical disc displacement, mid-cervical region, unspecified level - M50.220
14. Other cervical disc displacement at C4-C5 level - M50.221
15. Other cervical disc displacement at C5-C6 level - M50.222
16. Other cervical disc displacement at C6-C7 level - M50.223

Treatment

1. **Radiculopathy, lumbar region**

   Start Celecoxib Capsule, 200 MG, 1 capsule with food, Orally, twice a day, 30 days, 60 Capsule, Refills 0
   Start Cyclobenzaprine HCl Tablet, 10 MG, 1 tablet as needed, Orally, twice a day, 30 days, 60 Tablet, Refills 0
   PROCEDURE: Lumbar Transforaminal Epidural w/o Sedation (Performed Date - 11/12/2024)

   > Notes: Harper, Fitzclarence N 11/11/2024 03:27:17 PM EST > Given positive exam findings, numbness and weakness along the L4/5, L5/S1 nerve roots paired with urinary frequency we will proceed with bilateral L4/5, L5/S1 transforaminal epidural steroid injections

   Notes: Pt will benefit bilateral L4/5& L5/S1 transforaminal epidural steroid injections to treat symptoms of significant nerve root compression with myelopathy in the presence of positive MRI findings.
2. **Sacroiliitis, not elsewhere classified**

Notes: He will also benefit rom bilateral Si Joint injections to decrase pain, increase low back mobility and to further facilitate therapy.

3. **Radiculopathy, cervical region**

   Notes: Pt will also benefit from cervical epidural treatment to treat symptoms of upper extremity nerve root compression, after aboe injections.
4. **Spondylosis without myelopathy or radiculopathy, lumbar region**

   Notes: If axial pain is persistent, will plan diagnostic medial branch blocks and consider radiofrequency treatmetn if it is indicated.
5. **Spondylosis without myelopathy or radiculopathy, cervical region**

   Notes: If axial pain is persistent, will plan diagnostic medial branch blocks and consider radiofrequency treatmetn if it is indicated.
6. **Sprain of ligaments of cervical spine, initial encounter**
   PROCEDURE: Trigger point injection (Performed Date - 11/12/2024)

   Notes: Harper, Fitzclarence N 11/11/2024 03:28:13 PM EST
   > Bilateral traps/rhombs

   Notes: Continue chiro per protocol.

7. **Sprain of sacroiliac joint, initial encounter**

   Notes: Coninue therapy.
8. **Sprain of ligaments of lumbar spine, initial encounter**

   Notes: Continue chiro per protocol
9. **Postconcussional syndrome**

   Notes: RiverMead Questiinnaire completed. Pt will likely need additional testing for concussion.
10. **Post-traumatic headache, unspecified, not intractable**

    Notes: If headaches are not relieved by above, will consider Sphenopalatine Ganglion Nerve Blocks (SPG) and Bilateral Occipital Nerve Blocks (BONB).
11. **Others**

    Notes: 1. Meds prescribed for pain and muscle spasms.
    2. Pt will follow up for re-evaluation and to execute above plan.

Visit Codes

- 99205 Office Visit, New Pt., Level 5.

Follow Up

2 Weeks

MITCHELL, Gerard **DOB:** 03/28/1977 (47 yo M) **Acc No.** 15514 **DOS:** 11/11/2024

Electronically signed by Fitzclarence Harper , MD on 11/12/2024 at 11:18 PM EST

Sign off status: Completed

Verispine Stockbridge
1506 ROCK QUARRY RD
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Tel: 678-782-7999
Fax: 404-334-7274

Progress Note: Fitzclarence Harper, MD 11/11/2024

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Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1997

Provider: Mary Dearborn, DC
Date of Service: 10/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT
Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

PAST, FAMILY, SOCIAL HISTORY
Surgeries/Hospitalizations: covid hospitalization
Allergies: none
Medications: none
Prior conditions: non-hodgkins lymphoma: remission since 2022

Father: in good health
Mother: in good health

REVIEW OF SYSTEMS
EAR/NOSE/THROAT: Ringing in ears
HEMATOLOGY/LYMPH: Lymphoma
MUSCULOSKELETAL: Stiffness, Muscle pain, Neck pain, Stiff neck and Back pain
NEUROLOGICAL: Headaches and Tingling
GENERAL: Loss of sleep

The following are negative: ALLERGIC-IMMUNOLOGIC, CARDIOVASCULAR, CONSTITUTIONAL, ENDOCRINE, EYES, GASTROINTESTINAL, GENITOURINARY, PSYCHIATRIC, RESPIRATORY, SKIN and MEN'S HEALTH ISSUES.

Collision occurred on 10/18/2024 at approximately 3:02 PM.
They the rear driver's side of my vehicle.
Symptoms reported: neck, midback, lowback, headaches, chest, and right hand pain.

Mr. Mitchell was the driver of the vehicle, at the time of the collision. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were locked, and the car did not move. Mr. Mitchell was seated in the vehicle as a driver and was wearing a seat belt with the shoulder harness and with the headrest in up position. The vehicle is equipped with air bags and they did not deploy. Mr. Mitchell didn't see the impact coming. Upon impact, his head was looking ahead and he was thrown forward. There was head trauma. Mr. Mitchell remembers the collision happening.
The patient reports to this office due to injuries received in an MVC

OBJECTIVE

VITALS
Height: 70 in Weight: 235 lbs BMI: 33.7
Pulse rate: 97 BPM (relaxed)
Blood Pressure (seated): right -N/A left-159/102 mmHg

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum.

Tenderness

Left Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)

Right Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)
- Lower Arm (moderate-severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)

Right Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)
- Lower Arm (moderate-severe)

Trigger Points

Left Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)

Right Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)
- Pelvis (moderate-severe)
- Lower Arm (moderate-severe)

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	Positive	
Foraminal compression test[2]	Positive	Positive	
Shoulder depression test[3]	Positive	Positive	
Spinal percussion test-thoracic[4]	Positive	Positive	
Kemp's test[5]	Positive	Positive	
Straight leg raise[6]	Positive	Positive	
Milgram's test[7]	Positive	Positive	
Phalen's sign[8]	WNL	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[4]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[5]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[6]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[7]Lower back pain may indicate a herniated disc.

[8]Tingling to the thumb and lateral 2 and a half digits may indicate carpal tunnel syndrome.

REFLEXES

Biceps, Tricep, Brachioradialis, Patellar and Achilles are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	40° (80% of normal)	Pain, Spasm, Guarding	
Extension	50° (83% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	35° (77% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	35° (77% of normal)	Pain, Spasm, Guarding	
Left rotation	70° (87% of normal)	Pain, Spasm, Guarding	
Right rotation	70° (87% of normal)	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	50° (83% of normal)	Pain, Spasm, Guarding	
Extension	15° (60% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	15° (60% of normal)	Pain, Spasm, Guarding	
Right ateral flexion	15° (60% of normal)	Pain, Spasm, Guarding	
Left rotation	20° (66% of normal)	Pain, Spasm, Guarding	
Right rotation	20° (66% of normal)	Pain, Spasm, Guarding	

Wrist ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	L: R: 80° (88% of normal)	Pain, Spasm, Guarding	
Extension	L: R: 60° (85% of normal)	Pain, Spasm, Guarding	
Abduction	L: R: 20° (80% of normal)	Pain, Spasm, Guarding	
Adduction	L: R: 55° (84% of normal)	Pain, Spasm, Guarding	

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL. visible redness over left pectoralis muscle/sternum..
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

There are no contraindications to chiropractic care. Discussed high blood pressure and referred to PCP for further evaluation and treatment.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)

15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.
- Promote soft tissue healing.
- Relieve pain and prevent recurrence.
- Increase pain-free ROM.
- Be able to perform normal ADL's.
- Reduce headaches.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings. The current complaints are a result of this MVC.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

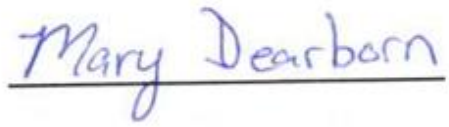
- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain

reported at initial encounter by 50%.

- New Patient evaluation and management service, 99203, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 4 weeks.

Patient received care without incident.



(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 10/22/2024 04:19 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Jessie L. Stewart, DC
Date of Service: 10/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum.

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T2; T6; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Diagnostic Tests: X-ray cervical 2-3 views (72040) was provided due to significant traumatic mechanism of injury.**
- **Diagnostic Tests: X-ray thoracic 2 views (72070) was provided due to significant traumatic mechanism of injury.**
- **Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided due to significant traumatic mechanism of injury.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Jessie L. Stewart, DC
Date: 10/25/2024 05:04 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 10/28/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he feels about the same today.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; **T1**; **T2**; **T3**; **T6**; **T7**; **T8**; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of **ice pack** was provided specifically to **Neck**, to **reduce pain reported at initial encounter by 25%**.
- **Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.**
- **Established Patient evaluation and management service, 99212, was provided today.**

X-ray Results were reviewed today with the patient. The patient had the opportunity to ask questions and understand the findings.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 10/28/2024 04:31 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 10/29/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he feels about the same today.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum.

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Prognosis has changed from **Guarded to Fair** - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 10/29/2024 03:46 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Amanda Ruiz, DC
Date of Service: 10/31/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum.

Tenderness

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/31/2024 03:43 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: **From 7 to 6/10 (10% decrease)**
  Described as **frequent (up to 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 7/10
  Described as **frequent (up to 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as **frequent (up to 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: **From 7 to 6/10 (10% decrease)**
  Described as **frequent (up to 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as **frequent (up to 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says his pain is a little less frequent.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine **(decreased 5-10% to moderate)**
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine **(decreased 5-10% to moderate)**
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine **(decreased 5-10% to moderate)**

Right Side

- Cervical Spine **(decreased 5-10% to moderate)**
- Shoulder **(decreased 5-10% to moderate)**
- Thoracic Spine **(decreased 5-10% to moderate)**

- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 11/04/2024 04:14 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 6/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: **From 7 to 6/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: **From 6 to 5/10 (10% decrease)**
  Described as frequent (up to 75% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 8/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says **he feels he** is **slowly improving**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

Trigger Points

Left Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)

Right Side

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate-Severe)
- Pelvis (Moderate-Severe)
- Lower Arm (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 11/05/2024 03:53 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/7/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 6/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 6/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: **From 7 to 6/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as intermittent (up to 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: **From 8 to 6/10 (20% decrease)**
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he feels **a little better today**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Thoracic Spine **(decreased 5-10% to moderate)**
- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Cervical Spine (Moderate)
- Shoulder (Moderate)

Right Side

- Thoracic Spine **(decreased 5-10% to moderate)**
- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Lower Arm **(decreased 5-10% to moderate)**
- Cervical Spine (Moderate)
- Shoulder (Moderate)

Spasm/Hypertonicity

Left Side

- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

Right Side

- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Lower Arm **(decreased 5-10% to moderate)**
- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

Trigger Points

Left Side

- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Cervical Spine (Moderate)

Right Side

- Lower Back **(decreased 5-10% to moderate)**
- Pelvis **(decreased 5-10% to moderate)**
- Lower Arm **(decreased 5-10% to**

- Shoulder (Moderate)
- Thoracic Spine (Moderate)

moderate)

- Cervical Spine (Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 11/07/2024 04:25 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: **From 6 to 5/10 (10% decrease)**
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 6/10
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 6/10
  Described as **intermittent to frequent (up to between 50% and 75% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: **From 5 to 3/10 (20% decrease)**
  Described as **intermittent (up to 50% of day)** of **slight intensity (requires some modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 6/10
  Described as **intermittent (up to 50% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he **has noticed slight improvement**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder **(decreased 5-10% to mild-moderate)**
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder **(decreased 5-10% to mild-moderate)**
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine **(decreased 5-10% to mild-moderate)**
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)

Provider Name: Mary Dearborn, DC
Date: 11/11/2024 04:21 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 6/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 3/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 6/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he **feels okay today**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)

2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99212, was provided today.**

MRI Results were reviewed today with the patient. The patient had the opportunity to ask questions and understand the findings.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)

Provider Name: Mary Dearborn, DC
Date: 11/15/2024 05:17 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 11/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 5/10
  Described as **intermittent (up to 50% of day)** of **moderate intensity (requires significant or complete modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 3/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: **From 6 to 5/10 (10% decrease)**
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Shoulder (Moderate)
- Thoracic Spine (Moderate)
- Lower Back (Moderate)
- Pelvis (Moderate)
- Lower Arm (Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)

3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 11/18/2024 04:57 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Amanda Ruiz, DC
Date of Service: 11/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: **From 5 to 4/10 (10% decrease)**
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: **From 5 to 4/10 (10% decrease)**
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 3/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 5/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

VITALS

Height: 70 in Weight: 235 lbs BMI: 33.7
Pulse rate: 97 BPM (relaxed)
Blood Pressure (seated): right -N/A left-165/115 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 10-20% to mild)**
- Pelvis **(decreased 10-20% to mild)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 10-20% to mild)**
- Pelvis **(decreased 10-20% to mild)**
- Lower Arm **(decreased 10-20% to mild)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 10-20% to mild)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 5-10% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 10-20% to mild)**
- Thoracic Spine **(decreased 10-20% to mild)**
- Lower Back **(decreased 5-10% to mild-moderate)**
- Pelvis **(decreased 5-10% to mild-moderate)**
- Lower Arm **(decreased 5-10% to**

mild-moderate)

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 10-20% to mild)**
- Thoracic Spine **(decreased 5-10% to mild-moderate)**
- Lower Back **(decreased 5-10% to mild-moderate)**
- Pelvis **(decreased 5-10% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Shoulder **(decreased 5-10% to mild-moderate)**
- Thoracic Spine **(decreased 5-10% to mild-moderate)**
- Lower Back **(decreased 5-10% to mild-moderate)**
- Pelvis **(decreased 5-10% to mild-moderate)**
- Lower Arm **(decreased 5-10% to mild-moderate)**

GAIT

Ambulation: Normal.

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Foraminal compression test[1]	Positive	Positive	
Shoulder depression test[2]	Positive	**WNL**	
Kemp's test[3]	**WNL**	Positive	
Straight leg raise[4]	Positive	**WNL**	
Milgram's test[5]	Positive	Positive	
Phalen's sign[6]	WNL	Positive	

[1]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[2]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[3]Perhipheral pain in to the lower extremities indicates nerve root compression. Local pain may indicate sprain or strain.

[4]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[5]Lower back pain may indicate a herniated disc.

[6]Tingling to the thumb and lateral 2 and a half digits may indicate carpal tunnel syndrome.

Cervical distraction and Spinal percussion test-thoracic are within normal limits.

REFLEXES

Biceps, Tricep, Brachioradialis, Patellar and Achilles are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (90% of normal, increased by 12%)**	Pain, Spasm, Guarding	
Extension	**55° (91% of normal, increased by 10%)**	Pain, Spasm, Guarding	
Left lateral flexion	**40° (88% of normal, increased by 14%)**	Pain, Spasm, Guarding	
Right lateral flexion	**40° (88% of normal, increased by 14%)**	Pain, Spasm, Guarding	
Left rotation	**75° (93% of normal, increased by 7%)**	Pain, Spasm, Guarding	
Right rotation	**75° (93% of normal, increased by 7%)**	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**55° (91% of normal, increased by 10%)**	Pain, Spasm, Guarding	
Extension	**20° (80% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Left lateral flexion	**20° (80% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Right ateral flexion	**20° (80% of normal, increased by 33%)**	Pain, Spasm, Guarding	
Left rotation	**25° (83% of normal, increased by 25%)**	Pain, Spasm, Guarding	
Right rotation	**25° (83% of normal, increased by 25%)**	Pain, Spasm, Guarding	

Wrist ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	L: R: **85° (94% of normal, increased by 6%)**	Pain, Spasm, Guarding	
Extension	L: R: **65° (92% of normal, increased by 8%)**	Pain, Spasm, Guarding	
Abduction	L: R: **WNL**	Pain, Spasm, Guarding	
Adduction	L: R: **60° (92% of normal, increased by 9%)**	Pain, Spasm, Guarding	

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL. visible redness over left pectoralis muscle/sternum. .
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 50% of goal achieved.**
- Restore tolerance to normal activities of daily living: **Progressed from 0% to 50% of goal achieved.**
- Increase pain-free ROM: **Progressed from 0% to 50% of goal achieved.**

Patient is **showing slight improvement** since the prior visit, He is **progressing as anticipated**.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Reexam today. the new care plan will be 2 times per week for 4 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/25/2024 05:24 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Amanda Ruiz, DC
Date of Service: 11/26/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 4/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 3/10
  Described as intermittent (up to 50% of day) of slight intensity (requires some modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT

Right finger, Right hand, Right wrist

- Sharp pain: 5/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum.

Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)
- Lower Arm (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)

3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

The patient received injections on the neck region at the ortho appointment today.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/26/2024 05:41 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 12/5/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 4/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: **From 3 to 2/10 (10% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of **minimal to slight intensity (symptom is not alleviated by any amount of modification**

to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT
Right finger, Right hand, Right wrist

- Sharp pain: 5/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he has noticed improvement but his pain still comes and goes.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)
- Lower Arm (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)

Right Side

- Cervical Spine (Mild)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Post-traumatic headache, unspecified, not intractable (G44.309)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Pain in joints of right hand (M25.541)
14. Chest pain, unspecified (R07.9)
15. Muscle spasm of back (M62.830)
16. Sleep deprivation (Z72.820)
17. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; **Sacrum**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of ice pack was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

Mary Dearborn

(Electronic Signature)
Provider Name: Mary Dearborn, DC

Date: 12/05/2024 05:29 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Gerard Mitchell
Date of Birth: 3/28/1977

Provider: Mary Dearborn, DC
Date of Service: 12/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Sharp pain: 4/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

SECOND COMPLAINT
Upper back, Mid back

- Sharp pain: 5/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

THIRD COMPLAINT
Right buttock, Left buttock, Low back

- Pain: 4/10
  Described as intermittent to frequent (up to between 50% and 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

FOURTH COMPLAINT
Head

- Aching, Throbbing pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity.

FIFTH COMPLAINT
Left chest

- Aching pain: 2/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by changing positions and ADL's; decreased by rest.

SIXTH COMPLAINT
Right finger, Right hand, Right wrist

- Sharp pain: **From 5 to 4/10 (10% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Says he has noticed **small improvements in** his **symptoms**

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; T1; T2; T3; T6; T7; T8; T9; L3; L5; Sacrum**.**

Tenderness

Left Side
- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)

Right Side
- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)
- Pelvis (Mild)
- Lower Arm (Mild)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)

Right Side
- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Shoulder (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)

Right Side
- Cervical Spine (Mild)
- Shoulder (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)
- Pelvis (Mild-Moderate)
- Lower Arm (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

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PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

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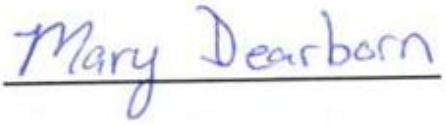
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Mary Dearborn, DC
Date: 12/06/2024 05:43 PM